Participant Profile Form

MSCA Staff Exchanges 2027 (MSCA-SE 2027) — consortium-building process

Unidad de Excelencia IOBA | University of Valladolid

Purpose:
This form is intended to collect structured information from interested participants for the preparation of an MSCA Staff Exchanges consortium. It is designed to support both (i) later completion of participant-related information requested in the EU Funding & Tenders Portal, and (ii) the creation of an internal profile database to help identify complementarities and facilitate contacts across the emerging consortium.

Please complete one form per:

- non-academic organisation (SMEs, NGOs , patient organisations)
- participating academic team (one form per PI, research-group, department).

This prospective **MSCA Staff Exchanges 2027 consortium** aims to build an international, interdisciplinary and inter-sectoral network in eye and vision research, centred on translational collaboration and staff mobility. Starting from Unidad de Excelencia IOBA's strengths in advanced therapies, biomarkers, inflammation, neuroprotection and health technology assessment, the network remains open to additional interdisciplinary collaborations, including areas such as ocular surface, retina, inherited and age-related eye disease, neuro-vision, optometry, drug delivery, AI-enabled diagnosis and patient-oriented validation, while remaining fully open to any other complementary expertise that could enrich the consortium and strengthen its patient-focused impact.

Important:
- please use this form to capture your general interests, expertise, and rough capacity to send and/or host staff.
- If you are not sure how to answer any field, you may leave it blank and highlight it.

1. Organisation and administrative profile

Organisation legal name	University College Dublin
Short name / acronym used in projects	NUID
Participant Identification Code (PIC), if available	999974359
Website(s)	www.ucd.ie https://people.ucd.ie/alison.reynolds
Country	Ireland
Organisation type	**X** Academic ☐ Non-academic ☐ Hospital / clinic ☐ SME ☐ Large company ☐ Research infrastructure ☐ NGO / foundation ☐ Public body ☐ Other
Main address	University College Dublin, Belfield, Dublin 4, D04 W6F6.
Department(s), institute(s), research group(s) or unit(s) that would be involved	UCD School of Veterinary Medicine (Veterinary Biosciences Section) UCD Conway Institute of Biomedical & Biomolecular Research
Additional institutional resources relevant to collaboration	e.g. Cat 3 lab, Cat 2 facilities, transcriptomics facilities, tissue culturing, veterinary hospital (if clinicians involved), organoid facility, genomics core, proteomics core, microscopy core, biomedical facility (rodent and fish).

Main contact person for this profile	Alison Reynolds
Main contact email	alison.reynolds@ucd.ie

Other contact person(s), if relevant	N/A

2. Scientific / innovation profile and relevance for an IOBA-led consortium

Please describe your profile at the level of the participating team / unit most relevant to the prospective consortium.

Indicative thematic relevance (tick all that apply)	
X Ocular surface	**X** Retina
X Inherited retinal dystrophies (IRDs)	**X** Age-related ocular disease
☐ Neuro-vision / integrated vision / neuro-ophthalmology	☐ Optometry / visual behaviour / rehabilitation
☐ Advanced therapies / regenerative medicine	☐ Gene therapy / cell therapy / tissue engineering
X Inflammation / biomarkers / neuroprotection	**X** Drug delivery / formulations / nanomedicine
X Drug discovery / screening / repurposing	**X** Ocular vascular disease
☐ Infection / trauma-related ocular disease	☐ Ocular pain / quality of life / patient-reported outcomes
☐ Health technology assessment	☐ AI-assisted diagnosis / software / digital tools
☐ Clinical trials / translational validation	**X** Other (please specify): Public Patient Involvement, Surveys, Charles Bonney Syndrome

Free Keywords for internal searchable database (please provide up to 10)	Retinal (photoreceptor cell death, angiogenesis) and ocular surface (dry eye – inflammation, oxidation stress) disease models: zebrafish, organoid, mouse, *in vitro* (2D and 3D ocular surface cultures), drug discovery and development, space associated neuro-ocular syndrome (SANS),
Geographies of interest for exchanges (optional)	

Overview of your research / innovation focus

Please describe the main focus of the team / unit that could participate. Keep it concise but specific (recommended: 150-250 words).

Currently, the main focus is ocular surface: development of ocular surface disease models, ocular surface organoids, spheroids, stratified models, nanoformulated drug delivery (and discovery, development with “OcuHealth” in South East Technological University). We are also using zebrafish to see if we can develop a SANS model.

Why your team could be relevant to a translational eye-health and vision consortium

Please explain the specific added value your organisation / team could bring to a consortium centred on visual health, translational ophthalmology and related enabling technologies. ***Please think broadly: the contribution does not need to be directly ophthalmology-related. Interdisciplinarity is strongly encouraged, and expertise from apparently distant fields may open unexpected routes to innovation, new paradigms, and creative solutions to complex challenges that currently limit progress from research to patient benefit****.*

Expertise in both front of the eye and back of the eye disease modelling, understanding pathophysiology and drug discovery and development. This has covered inherited retail degeneration, angiogenesis, drug-induced models, together with a move to ocular surface disease modelling and understanding of disease progression, together with drug development and delivery technologies (with SETU). Commercialisation expertise (patenting, future spin out). PPI involvement (paper written to identify the gaps), survey design experience (Charles Bonnet Syndrome), also I am a board member of Fighting Blindness Ireland Charity.

Currently exploring how to model space-associated neuro-ocular syndrome.
I am also Associate Dean for Research, Innovation and Impact in the School of Veterinary Medicine, so keen to promote cross-discipline collaborations between our staff and graduate students.

3. Expertise, assets and collaboration interests

Core expertise, methods and platforms you could contribute

Please provide bullet points covering your strongest capabilities, methods, models, analytical approaches, translational assets or innovation strengths.

Very keen to collaborate to build better front of the eye models, in which we can learn more about cell communication, flare ups, and use these to design better drugs and drug-delivery methodologies.

Infrastructure, equipment, facilities or datasets relevant to consortium work

Examples: specialised laboratories, clean rooms, imaging platforms, -omics, biobanks, clinical cohorts, animal models, software tools, rehabilitation platforms, regulatory support or prototyping capacities.

Listed above: Cat 3 lab, Cat 2 facilities, transcriptomics facilities, tissue culturing, veterinary hospital (if clinicians involved), organoid facility, genomics core, proteomics core, microscopy core, biomedical facility (rodent and fish).
Datasets generated on novel early onset inherited retinal disease models.

Training capacity and knowledge-transfer potential

Please indicate what inbound staff could realistically learn in your organisation (scientific, technical, clinical, translational, regulatory, entrepreneurial or managerial skills).

Courses through the University (in multiple things- e.g. targeted graduate courses re specific research skills- proteomics, big data), technology transfer courses (entrepreneurial skills), innovation academy courses, research integrity courses. If wanted to pay, we have a professional academy with courses e.g. leadership & management.

What your team would hope to gain from participating

Examples: access to methods, models, technologies, patient-oriented validation, translational pathways, industrial insight, clinical exposure, data science, regulatory know-how, joint publications or longer-term strategic collaboration. ***Again, please think broadly and beyond ophthalmology.***

Building collaborative networks to learn more about how diseases progress.
Support in developing patient-relevant cures.
Helping facilitate more cross-EU contact between people with lived experience of blindness.
I co-chair the Irish National Animal Welfare Body (also we have just joined the European Network of National Animal Welfare Bodies ENAWB- I know Spain is also a member), so I would hope we could incorporate new ways to reduce, replace, refine animal use (and ex vivo animal part use) in the area.

4. Key people likely to be involved

Please list the key people who may be relevant to the consortium at this stage. *This is not a final, binding or secondment list.*

Name	Position / role	Department / unit	Staff category	Career stage	Email / identifier (optional)
Alison Reynolds	PI/Assistant Professor	School of Veterinary Medicine	Permanent/Academic	Mid	alison.reynolds@ucd.ie
Katarina Krsteva	MSCA Visiting Researcher	School of Veterinary Medicine	Visiting	Early	

Main disciplines represented in your team	Genetics, pharmacology, anatomy, physiology, embryology, commercialization, cell biology, molecular biology, animal expertise.
Experience in EU / international / multi-partner collaborative projects	I have been a PI on several EU RISE awards (3DNeoNet, Crystal3, Everest). I have participated as a work package manager and supervising incoming staff/students. I am currently involved with a cost Action (resubmission).
Experience with clinical, translational or patient-facing research (if relevant)	X Yes ☐ No ☐ Not applicable | Short note: Surveys of patients (including ethics writing), patient engagement days, board member on patient charity, published paper on PPI in vision research https://doi.org/10.12688/openreseurope.16057.1

5. Evidence of capacity and track record

Up to 5 relevant publications / datasets / software / products / services / other achievements

Please prioritise achievements most relevant to your possible contribution. Please include DOI, URL or identifier.

I collaborated with drug delivery experts and obtained Commercialisation Funding (CF-2020-1511; €700,000) to develop novel nanoformulations (SETU) with novel drugs (UCD) for dry eye disease, pitching and winning UCD's VentureLaunch Accelerator competition (2025) and came 3rd in Retinnovate's pitch competition (2026). A High Potential Start-Up will be spun out in 2026 to develop and commercialise this work to first in human.
(DOI: 10.1039/d4na01086h)

I worked with collaborators on a H2020-funded Innovative Training Network to Explore Public/Patient Involvement and challenges in visual impairment research (doi:10.12688/openreseurope.16057.1)

I supervised PhD students who developed models of stargardt disease in zebrafish (DOI: 10.1007/978-3-031-76550-6_36) and early onset severe retinal degeneration in organoids (https://doi.org/10.3389/fnins.2021.784987)

With a colleague in the Biomedical facility, we gained funding (2025-26) and successfully ran *Mental Health First Aider* and *Mental Health in the Workplace* training for staff and students working with animals. This included a bespoke model on "Compassion fatigue" for those working with animals.

Up to 5 relevant previous projects, grants, networks or collaborations

Please provide detailed information (Title, Acronym, funding agency, GA-number, dates). Please indicate your role (coordinator / beneficiary / partner / service provider / collaborator). ***Indicate if international.***

1. OcuHealth: Enterprise Ireland Commercialisation Funding (CF-2020-1511; €700,000). Co-PI. National (2021 – 2026)
2. CRYSTAL3; MSCA/HE grant agreement, 101007931. Work Package leader, collaborator. International (2021 – 2024)
3. EVEREST MSCA/HE GA 101183034. Supervisor of visiting researchers. International (current)

Patents, protected know-how, clinical protocols, validated tools or other translational assets (if any)
Please indicate only what can be shared at this stage. Previous patents awarded: USPTO and EPO (2 each), antiangiogenic compounds.

6. Staff-exchange capacity (only rough estimation at this stage)

Please provide only a rough, non-binding estimate of your organisation's/team's capacity to send and/or host staff **over the 48-month duration of the project.**

Could your organisation/team second staff?	☐ Yes **X** Maybe ☐ No capacity to send staff on secondment
Could your organisation second staff categories beyond researchers (where relevant to R&I activities)?	☐ Only Researchers **X** Technical Staff/Innovation **X** Managerial / project support **X** Admin Staff ☐ Other profiles **X** Maybe
Could your organisation host incoming staff from academic and/or non-academic sectors?	☐ Academic only ☐ Non-academic only **X** Both ☐ No capacity to host
Administrative / HR / legal considerations that may affect mobility	If EU/ UK passport and citizen- this is fine. English language used in the lab- and staff seconding need to be competent to use on a daily basis. Health and Safety induction performed locally.

7. Matchmaking information: Summary for the internal participant database

Capabilities you would most like other interested participants to notice
Disease modelling *Commercialisation interests* *Patient-centric support*

Capabilities or expertise you would especially like to access through the consortium
Please indicate the types of teams, methods, infrastructures or translational pathways you would like to connect with in the consortium. New methods for analysis of front of the eye disease models Development of new cures Bringing patients together

Please Confirm:

Open to being contacted by other interested participants through the coordination team's mediation?	X Yes ☐ No
X We understand that the information contained in this form will be searchable and accessible to other interested participants within the pre-consortium process.	

8. Final remarks and query items

Please note any uncertainties, prerequisites, internal approvals needed, or items that should be considered further before the consortium is consolidated.
Examples: PIC pending approval, institutional approval, legal status, hosting rules, IP constraints, data access, ethics, clinical permissions, staffing continuity, resource availability. Ethics requirements can take some time (Human or animal ethics) – so if a specific project is required, planning may be required far in advance. DPIA agreements may be required. Limited availability to send out secondees. Would require internal approval to send PI on secondment (unlikely to happen prior to mid 2028, due to leadership role).

9. Additional collaborators you would like us to consider

If there are any additional organisations, teams or contacts - current collaborators, previous collaborators or desired future collaborators - that you think could be relevant for this MSCA-SE consortium, please list them below. The coordination team may contact them to explore their potential interest in joining the consortium-building process.

Please provide the following details (as many entries as needed).

Type of organisation: ☐ Academic ☐ Clinical ☐ SME/industry ☐ NGO/foundation ☐ Public body ☐ Other

Organisation / team name:

Country:

Website:

Existing link with your team: ☐ Current collaborator ☐ Previous collaborator ☐ Desired collaborator ☐ Other

Suggested contact person/email (if known):

Why could they be relevant for this MSCA-SE consortium?

Any additional comments or sensitivities before contacting them?

Internal use and confidentiality statement

The information provided in this form will be used only for internal consortium-building purposes in connection with the preparation of a prospective MSCA Staff Exchanges proposal (2027 call) coordinated from Unidad de Excelencia IOBA / University of Valladolid. It may be used by the coordination team to: (i) assess strategic fit and administrative readiness, (ii) prepare participant-related information required at proposal stage, and (iii) build an internal profile database to facilitate contacts among interested participants. The information will not be shared outside this pre-consortium context without prior permission from the contributing organisation / contact person, except where later required for the formal preparation of a proposal with the explicit involvement of that organisation. Submission of this form confirms that the contributor agrees to this internal use. Organisations may request correction, update or withdrawal of their information at any time by contacting the coordination team.

X Understood

Name: Alison Reynolds
Date completed: 24th September 2026

(Coordination team: ivan.fernandez.bueno@uva.es; yolanda.alvarez.teruel@uva.es)